

Antepartum Hemorrhage (APH)

1. Definition

Antepartum hemorrhage is defined as bleeding from the genital tract after 28 weeks of pregnancy and before the birth of the baby.

It is a serious obstetric complication that can affect both mother and fetus.

2. Types of Antepartum Hemorrhage

1. Placenta Previa

2. Abruptio Placentae (Placental Abruption)

Other causes may include:

- Trauma to genital tract
- Rupture of uterus
- Cervical lesions or infections

Placenta Previa

Definition

Placenta previa is a condition where the placenta is partially or completely implanted in the lower uterine segment and covers the cervical opening (internal os).

Causes / Risk Factors of Placenta Previa

- **Previous cesarean section**
- **Multiple pregnancy**
- **Advanced maternal age**
- **High parity (many previous births)**
- **Previous placenta previa**
- **Smoking**
- **Abnormal uterus**
- **Large placenta**
- **Previous uterine surgery**

Difference Between Placenta Previa and Abruptio Placentae

Feature	Placenta Previa	Abruptio Placentae
Definition	Placenta attached in lower uterus	Placenta separates from uterus before delivery
Bleeding	Painless bleeding	Painful bleeding
Blood color	Bright red	Dark red
Uterine pain	Usually absent	Severe abdominal
Uterus	Soft and relaxed	Hard and tender

Fetal
condition

Usually normal
initially

Fetal distress com

Shock

Less severe

More severe

Presentation

Often
malpresentation

Usually normal
presentation

Nursing Management of Placenta Previa

1. Assessment

- Monitor **vital signs** (pulse, BP, respiration).
- Check **amount and color of bleeding**.
- Monitor **fetal heart rate**.
- Assess **uterine contractions**.
- Check signs of **shock**.

2. Bed Rest

- Advise **strict bed rest**.
 - Place mother in **lateral position** to improve circulation.
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3. Avoid Vaginal Examination

- **Digital vaginal examination should not be done** because it can cause severe bleeding.

4. Maintain IV Line

- **Start intravenous fluids.**
 - **Prepare for blood transfusion if required.**
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5. Monitor Fetal Condition

- **Regular fetal heart monitoring.**
- **Observe for fetal distress.**

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6. Prepare for Delivery

- **If bleeding is severe, prepare for emergency cesarean section.**
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7. Emotional Support

- **Provide psychological support to the mother and family.**
- **Explain procedures to reduce anxiety.**

Types of Placenta Previa

Placenta previa is classified according to how much the placenta covers or reaches the cervical opening (internal os).

1. Type I – Low-lying Placenta

- Placenta is located in the lower uterine segment.**
- It does not reach the cervical opening (internal os).**
- Usually causes little or no bleeding.**
- Normal vaginal delivery is often possible.**

4. Type IV – Complete Placenta Previa

- **Placenta completely covers the internal os.**
- **Severe bleeding occurs.**
- **Normal delivery is not possible, and cesarean section is necessary.**

Abruptio Placentae (Placental Abruption)

1. Definition

Abruptio placentae is a condition in which the normally situated placenta separates partially or completely from the uterine wall before the birth of the baby.

It is a serious complication that can cause heavy bleeding and danger to both mother and fetus.

Risk Factors of Abruptio Placentae

- **Hypertension (high blood pressure)**
- **Pre-eclampsia**
- **Trauma or injury to abdomen**
- **Previous history of placental abruption**
- **Smoking**
- **Alcohol or drug use**
- **Multiple pregnancy**
- **Advanced maternal age**
- **Sudden loss of amniotic fluid**
- **Short umbilical cord**

Signs and Symptoms

- Sudden vaginal bleeding
- Severe abdominal pain
- Back pain
- Uterine tenderness
- Rigid or hard uterus
- Decreased fetal movement
- Fetal distress
- Signs of shock (low BP, rapid pulse)

Clinical Manifestations

- Painful vaginal bleeding
 - Board-like (hard) uterus
 - Frequent uterine contractions
 - Maternal shock
 - Fetal distress or fetal death
 - Anemia
 - Coagulation disorders (DIC in severe cases)
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3. Bed Rest

- Keep the mother in **left lateral position** to improve blood flow.
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4. Oxygen Therapy

- Provide **oxygen** to improve fetal oxygen supply.
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5. Continuous Monitoring

- Monitor **uterine contractions**.
- Check **fetal condition regularly**.

Management of Abruptio Placentae

1. Immediate Assessment

- Check maternal vital signs (BP, pulse, respiration).
 - Monitor fetal heart rate.
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2. Control Bleeding

- Insert IV line and start IV fluids.
 - Prepare for blood transfusion if necessary.
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Types of Abruption Placentae

1. Revealed Hemorrhage

- Blood comes out through the vagina.
- Vaginal bleeding is visible.
- Easier to diagnose.

2. Concealed Hemorrhage

- Blood remains inside the uterus behind the placenta.
- Little or no vaginal bleeding is seen.
- More dangerous because bleeding is hidden.

3. Mixed Hemorrhage

- Some blood comes out vaginally and some remains inside the uterus.

2. Type II – Marginal Placenta Previa

- Placenta reaches the margin of the internal os but does not cover it.
 - May cause mild bleeding during labor.
 - Vaginal delivery may still be possible in some cases.
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3. Type III – Partial Placenta Previa

- Placenta partially covers the internal os.
- Bleeding is more severe.
- Usually requires cesarean section for delivery.